



Personality disorder

Individuals who display persistent inappropriate, inflexible, or extraordinary behaviors, or have problems relating to others, have a personality disorder, or PD. There are several PD types, ranging from antisocial (BPD) to schizotypal, but some sufferers can manage their lives without medical help.

A personality disorder involves a consistent pattern of behavior that deviates noticeably from that which is considered acceptable by society. Symptoms usually appear by adolescence and can lead to long-term difficulties for sufferers, in terms of navigating relationships and simply functioning effectively in social situations.

The many types of PDs are broadly grouped into three groups or “clusters”: suspicious; emotional and impulsive; and anxious (see box, below). Each type has its own symptoms. For example, a person with a suspicious personality disorder is typically antisocial, easily frustrated, and has difficulty controlling anger. Borderline personality disorder (BPD)—a type of emotional and impulsive PD—is associated with disturbed ways of

thinking, impulsive behavior, and problems controlling emotions.

The anxious cluster includes avoidance personality disorder, which is characterized by feelings of inadequacy and extreme sensitivity to negative criticism and rejection. Unsurprisingly, people who have this type of PD also experience severe social anxiety.

The PD brain

Some people affected by PDs have an unusual amygdala, part of the limbic system—the most primitive part of the brain that regulates fear and aggression. People who have PDs involving excessive levels of fear generally have smaller amygdalae than those who do not, and the smaller the amygdala, the more overactive it seems to be. In addition, the hippocampus,

75 PERCENT
OF PEOPLE
DIAGNOSED
WITH BPD
ARE WOMEN



which also helps control emotions, is often reduced in the brain of individuals with PDs.

People with PDs usually find that talking therapies help them gain a better understanding of their thoughts, feelings, and behaviors. Therapeutic communities, a form of group-therapy treatment, can also be effective but require a high level of commitment. Medication may also be used in some cases to control depression and anxiety.

PERSONALITY DISORDER CLUSTERS

CLUSTER A: SUSPICIOUS

People with these PDs tend to be considered odd or “eccentric.” They fear social situations and have problems relating to other people, whom they view with a great deal of suspicion. Some sufferers appear detached, others introverted.

CLUSTER B: EMOTIONAL AND IMPULSIVE

These PD types are characterized by a lack of emotional control. Cluster B individuals often bully or manipulate others, are self-centered, and are prone to dramatic, excessive displays, forming intense but short-lived relationships.

CLUSTER C: ANXIOUS

The most fearful cluster of PDs. Those in this group are generally anxious, submissive to others, and have difficulty coping with life on their own. They tend to be oversensitive, inhibited, extremely shy, or perfectionists.

Paranoid

Antisocial

Avoidant

Schizoid

Borderline

Dependent

Schizotypal

Histrionic

Obsessive-compulsive

Narcissistic